

CLL quick reference

v2.0 relapsed/refractory update

MHA-CLL-2026-v2.0

Evidence and access cut-off: 26 July 2026

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Clinical decision-support only. Not for direct patient use. Use current SmPCs, NICE guidance, NHS commissioning rules, local policy and patient-specific clinical judgement. England-specific NICE/NHS access must not be described as a single UK-wide entitlement.

Before any treatment line

- Treat only when iwCLL active-disease criteria are met.
- Review prior regimen, class, duration, depth of response, reason for stopping and treatment-free interval.
- Repeat del(17p) FISH and TP53 sequencing before the next treatment line.
- Assess rapidly progressive or asymmetric disease, disproportionate LDH rise or systemic deterioration for Richter transformation; use PET-CT-directed biopsy.
- Document fitness, cardiac and bleeding risk, renal function, TLS risk, infection burden, interactions, patient preference and access route.

First-line orientation

State	Route
del(17p) and/or TP53 mutation	Targeted therapy only. BSH 2025 favours zanubrutinib or acalabrutinib; TA931 and TA689 apply only within their specified untreated populations. Use current SmPC and Blueteq criteria.
No TP53 abnormality	Continuous zanubrutinib/acalabrutinib or fixed-duration venetoclax–obinutuzumab. Decide with the patient and apply the exact treatment-specific NICE criteria.

Relapsed/refractory treatment state

Definitions

State	Definition
Intolerant	Stopped for toxicity without progression. A better-tolerated agent within class may remain appropriate.
Exposed, not refractory	Completed or stopped treatment without progression; sensitivity may remain.
Refractory	Progression during active treatment or no meaningful response.
Double exposed	Both a covalent BTKi and venetoclax have been used.
Double refractory	Progression during both classes. Distinct high-risk state; not synonymous with double exposure.

Sequencing

Prior-treatment state	Decision route	Do not
Covalent-BTKi intolerance	Alternative better-tolerated covalent BTKi or different class according to comorbidity, preference and NICE eligibility.	Do not describe an intolerance switch as efficacy after resistance.
Covalent-BTKi progression; venetoclax naive	Venetoclax-based NICE pathway when eligible; pirtobrutinib under TA1173 when its restriction is met.	Do not continue covalent BTKi monotherapy as definitive resistant-disease treatment.
Relapse after fixed-duration venetoclax response	BTKi or selected venetoclax retreatment after reviewing prior response and treatment-free interval.	Do not extrapolate retreatment evidence to primary or on-treatment resistance.
Venetoclax progression; BTKi naive	NICE-funded covalent BTKi when eligible.	Do not claim a universal randomised sequencing hierarchy.
Double exposed, not double refractory	Individualise by retained sensitivity, prior response and access.	Do not automatically label double refractory.
Double refractory	Early tertiary/trial referral; pirtobrutinib if TA1173 criteria met; highly selected cellular therapy/alloHCT discussion.	Do not imply routine NHS England CLL CAR-T commissioning or established devolved-nation access, or a comparative hierarchy.

NICE recommendations and NHS England access at 26 July 2026

Access and schedule

Option	NICE position	Schedule / control
Venetoclax–rituximab	TA561 after ≥1 previous therapy.	Five-week venetoclax ramp-up to 400 mg OD before rituximab; rituximab 375 mg/m ² C1D1 then 500 mg/m ² D1 C2–6; venetoclax for 24 months from C1D1 rituximab.
Venetoclax monotherapy	TA796: (a) del(17p)/TP53-mutated CLL when a B-cell receptor pathway inhibitor is unsuitable, or after progression on one; or (b) CLL without del(17p)/TP53 mutation after progression following both chemoimmunotherapy and a B-cell receptor pathway inhibitor.	Five-week ramp-up to 400 mg OD; continue until progression or unacceptable toxicity.
Acalabrutinib	TA689 for previously treated CLL and specified untreated populations.	100 mg BD continuously; current SmPC and Blueteq criteria.
Zanubrutinib	TA931 for R/R CLL and specified untreated populations.	160 mg BD or 320 mg OD continuously; current SmPC and Blueteq criteria.
Pirtobrutinib	TA1173: adults with R/R CLL who have had a BTKi, only if covalent-BTKi retreatment, including after a fixed-duration regimen, is not clinically appropriate.	200 mg OD continuously. At 26 July 2026 NHS England access remained through interim Blueteq funding; routine-budget implementation was due by 29 September 2026. The broader MHRA licence does not widen NHS funding.

Venetoclax safety: Apply current SmPC TLS stratification, prophylaxis, monitoring, renal-function assessment and interaction management. This card does not replace the prescribing information or local protocol.

Emerging or specialist options

- Pirtobrutinib–venetoclax–rituximab: phase III BRUIN CLL-322 evidence; NICE ID6566/GID-TA11748 awaiting development at the cut-off. Not routine NHS treatment.
- Lisocabtagene maraleucel: single-arm CLL evidence; no demonstrated CLL marketing authorisation, NICE recommendation or routine NHS England commissioning at the cut-off. Devolved-nation access was not established by this audit. NICE ID6174/GID-TA11001 discontinued February 2026.
- CD20×CD3 bispecific antibodies: no qualifying peer-reviewed prospective therapeutic trial in untransformed CLL identified through the cut-off. Do not extrapolate Richter or other-lymphoma evidence.

Release control

Published 27 July 2026. Independent review, pharmacy verification and clinical-owner authorisation are complete. Educational clinical decision support only; verify current NICE, NHS, SmPC and local policy before prescribing.